



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هدى فاروق محمد عبدالمنصف

العنوان :

*****/

المحافظة : الجيزة

رقم البطاقة : 27502042101181 رقم التليفون : 01140211146

تاريخ تسجيل الإستمارة : 20-09-2026

العمر(تاريخ الميلاد) : 04-02-1975 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك الإخصائي الاجتماعي

المقر بما فيه التوقيع: (-----)

التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعة مقطعية مخ مع صبغة [10851] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني

الاسم : د/ندى حسن

الاسم : د/نرمين رمضان

الوظيفة : -----

الوظيفة : -----

الوظيفة : -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي : -----

خطة العلاج : -----

المستشفى : -----

قسم : -----

جهة العلاج : -----

نفقات : -----

ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هدى فاروق محمد عبد المنصف National ID: 27502042101181	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Contrast-enhanced CT of the brain.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تعقب الشخصية

هادي
شاري محمد عبد المنصف
من شوقي الرئيس
القومية - الجوز

١٩٧٥/١٠/٢١
٢٧٥٠٢٠٤٢١٠١١٨١

LA7331704

٢٠٢٥/٣/٢٧
٢٧٥٠٢٠٤٢١٠١١٨١

ربة منزل

أنش
مسئمة
متروحة

محمد عبده محمد أحمد
البطاقة سارية حتى ٢٠٣٦/٣/٢٧

MDT FORM

4/12/2025 G

Name	أحمد محمد أحمد	MRN	٢٧٥٢/٢٢
Age	49y.	ID	٢٧٥٢/٢٢٢٢٢٢٢٢
Menopausal status	premen	Phone	01140211406
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDC	R	PS: 4 (L.L. weakness)
ER:	8/8	PR:	3/3
HER 2:	-	Ki67:	15%
Sonomammography:	premen MB		
Mets Workup:	2019		

Recommendations:

→ St. Aramian, Ibrance 12/2024 → 4/2025

Op. St. Kelada x 0.5g

Renty PD. clinically

L.L. weakness.

MDT OUTCOME & PLAN

BS: 29/9/2025, widespread ossa

MDT Biopsy - From # FL

SIGNATURE

Meanwhile Faval weekly
pt is symptomatic

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name: Hoda Farouk Mohamed Abd Al Monsef	Date : 18/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 3753/2024	Pat. ID: 97992

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Reported case of left breast cancer compared to last study dated 18/8/2025 the current study showed:

I Chest: -

- Left mastectomy showing clear operative bed ,no sizable masses.
- Mild progressive course regarding the size of one of the right upper lone pulmonary nodules measuring 5 mm compared to 3.5 mm in the previous study.
- No appreciable time interval changes regarding the rest of the previously seen few bilateral scattered small and tiny pulmonary nodules , largest is still seen at the left lower lung lobe measuring about 6-7mm.
- **Still noted** Few subcentimetric mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size showing rather stationary course regarding the size and number of the previously noted few bilobar hepatic focal lesions currently being multiple , The largest is still noted at segment VIII measuring 19 x 14 mm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries
- Average size of the spleen showing no focal lesions.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Still noted mesenteric area of fat stranding with multiple subcentimetric mesenteric lymph nodes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**

- **Mild progressive course regarding the size and number of the previously seen** diffuse axial and appendicular heterogenous osseous texture with wide spread innumerable lytic and sclerotic osseous deposits as well as the size of the right 2nd rib and L2 vertebral body extra osseous soft tissue components.
- Still noted L2 and D9 vertebral body wedging.

Opinion:

****status follow up showing :**

- Mild progressive course of the size of one of the right upper lobe pulmonary nodules, as well as the axial and appendicular lytic and sclerotic osseous deposits and extra osseous soft tissue component at right 2nd rib and L2 vertebral body.
- Rather stationary course regarding the rest of the study.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

Name : Huda Farouk Mohammed Abdelmonsef

Date : 29-09-2025 10:37

Branch : Dokki

Medical No. : 213592

Ref.Physician Prof : Gostaf Roseh Hospital

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 50-year-old lady, with left breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings:

- Widespread scattered foci of enhanced tracer uptake are seen at skull, whole spine, bilateral ribs, both humeri, scapulae, clavicles, sternum, pelvic bones, and both femora.

Impression:

- Widespread metastatic osseous lesions as described.
- Follow up bone scan for therapy monitoring is advised.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient Name: Hoda Farouk Mohamed Abd Al Monsef	Date: 18/08/2025
Exam: CT CAP+C (CT Chest Abd Pelvis)	Age: 50 Year(s)
Medical ID: 3753/2024	Pat. ID: 97992

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Reported case of left breast cancer compared to last study dated 27-2-2025.

I Chest: -

- Left mastectomy showing clear operative bed ,no sizable masses.
- Still noted bilateral scattered small and tiny pulmonary nodules , laregest is at left lower lung lobe anterior segment measuring about 6-7mm.
- Currently appreciated Few subcentemetric mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size showing mild regressive course of the previously noted few bilobar hepatic focal lesions currently being multiple , The laregest is currently noted at segment VII measuring 19 x 11 mm compared to last study noted at segment I measuring about 2.2 x1.9 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries
- Averges size of the spleen showing regression of well defined hypoodense focal lesion measuring about 12 x 9 mm compared to 2.4 x 2 cm priorly .
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**

- Still noted diffuse axial and appendiocular heterogenous osseous texture with wide spread innumerable sclerotic focal/patchy osseous deposites.
- L2 showing mild vertebral collapse.
- L5 central vertebral wedging.

Newly noted D9 central wedging

Opinion:

****status follow up showing :.**

- Mild regressive course of the size of bilobar hepatic focal lesions and splenic focal lesion.
- Newly noted D9 central wedging

Patient Name : الأستاذ / مدى فاروق محمد عبد المنصف
Patient No. : 27502042101181
Path No. : 4279/24
Sex : Female
Age : 49 Y
Referring Doc :
Request No. : 13004752
Request Date : 06/11/2024
Verification Date : 20/11/2024
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Known case of breast cancer with hepatic focal lesion.

Nature of Specimen: Tru-cut biopsy, collected at 5/11/2024.

Gross Examination:

- 4 grayish white tissue cores up to 2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Revealed hepatic tissue core infiltrated by malignant growth formed of hyperchromatic pleomorphic cells with increase N/C ratio.
- * IPT. (Positive control showed appropriate reactivity):
- GATA3: Strongly diffuse positive.
- ER: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
- PR: Weak nuclear reaction in 10% of tumor cells (score 3 /8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Hepatic focal lesion, Tru-cut biopsy:
Metastatic carcinoma of breast origin.

PT:

GATA3: Strongly diffuse positive.
ER: Positive (score 8 /8).
PR: Positive (score 3 /8).
Her2: Negative (score 0).

Verified by :

DR. Menna Nabil

Printing Date : 27/11/2024 09:32:43

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / هدى فاروق محمد عبد المنصف
Patient No. : 27502042101181
Path No. : 4279/24
Sex : Female
Age : 49 Y
Referring Doc :
Request No. : 13004752
Request Date : 06/11/2024
Verification Date : 20/11/2024
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.
Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Gamal Abdelatty.
Dr. Duaa Mohammed.

Verified by:
DR. Menna Nabil

ng Date : 27/11/2024 09:32:43

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